



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض رابعياً : جميله على حسين جادالكريم

العنوان : 50 ش محمد ابراهيم -
الجيزه

المحافظة : الجيزه

تاريخ تسجيل الإستمارة : 28-09-2026

رقم البطاقة : 25712072701644 رقم التليفون : 01126461974

العمر(تاريخ الميلاد) : 07-12-1957 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإحصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: جميله علي حسين جادالكريم National ID: 25712072701644	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

٢٠٢٦/٠٤ ٢٥٧١٢.٧٢٧.١٦٤٤
 ربة منزل
 أنثى مسلمة
 متزوجة
 محمد حامد حسين
 البطاقة سارية حتى ٢٠٣٣/٠٤/٠٨
 ١



جمهورية مصر العربية
 بطاقة تحقيق الشخصية
 جميله
 على حسين جاد الكريم
 ١ ش سامي ابومبروك المعتمدية
 مركز كرداسة - الجيزة
 ١٩٥٧/١٢
 ٢٥٧١٢.٧٢٧.١٦٤٤
 LR9164857



National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة



Pathology Report

Name: 3. جميله على حسين جاد الكريم Age: 64 years.
Clinical data: Bilateral breast masses.
Specimen type: Tru cut biopsy.
Date (Received): 3/7/2021. Date (reported): 14/7/2021 (IPT)
Lab. Number: 1409/ 21 (final) Hospital Number:

Gross Examination:

Two specimens were received and submitted as:

- (1) Right breast core biopsy; 0.2 cm up to 1.3 cm long. Totally submitted.
 - (2) Left breast core biopsy; grayish white, up to 0.5 cm. Totally submitted.
- Procedure: Tru cut
 - Localization: Right and left breast
 - Tumor extent; N/A
 - Number:
 - Size:
 - Skin involvement:
 - Distance from margins: N/A
 - Lymph nodes number and size: N/A
 - Additional pathologic findings: N/A
 - Immunohistochemistry technique:

Name: <u>جميلة علي حسين</u>		
MR. : <u>7280</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient is a post-menopausal woman with a history of invasive ductal carcinoma of the breast, stage T4B N1 M0, ER 8/8, PR 7/8, HER2 negative, who underwent a modified radical mastectomy followed by postoperative radiotherapy and is currently receiving adjuvant aromatase inhibitor therapy. In 2026 the patient presented with a left supraclavicular lymph node enlargement. Surgical consultation recommended a tru-cut biopsy, which revealed metastatic deposits of breast origin, grade 3, ER 8/8, PR 6/8, HER2 +1, Ki-67 25%. A metastatic work-up was initiated and a new multidisciplinary team (MDT) discussion was scheduled.		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	
	علياء احمد أبو النصر شهاب الدين	الطبيب المعالج :
يعتمد ,		

Microscopic examination:

- Histological type: (1) Right breast; Invasive duct carcinoma grade II. (2) Left breast, core biopsy; intramammary lymph node with reactive changes.
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent N/A
 - Number:
 - Size:
 - Skin involvement:
- Lymph node involvement: N/A
- Resection Margins: N/A
 - Free /DCIS/ invasive carcinoma
- Additional pathologic findings:

Diagnosis:

Bilateral breast masses, Core biopsies;

- Right breast, Core biopsy; Invasive duct carcinoma grade II.
- ER: Strong Intensity 90% nuclear reaction (Allred score 8/8)
- PR: Moderate Intensity 90% nuclear reaction (Allred score 7/8)
- Her2: No membrane reaction (score 0)
- Ki67: 10% nuclear reaction
- Left breast, Core biopsy; Intramammary lymph node with reactive change.

Prof. Dr. Lubna Omar.

Dr. Mona Mohamed



Pathology Report

Name: جميله على حسين جاد الكريم 3. Age: 64 years.
Clinical data: Bilateral breast masses.
Specimen type: Tru cut biopsy.
Date (Received): 3/7/2021. Date (reported): 7/7/2021
Lab. Number: 1409/ 21 Hospital Number:

Gross Examination:

Two specimens were received and submitted as:

- (1) Right breast core biopsy; 0.2 cm up to 1.3 cm long. Total submitted.
 - (2) Left breast core biopsy; grayish white, up to 0.5 cm. Total submitted.
- Procedure: Tru cut
 - Localization: Right and left breast
 - Tumor extent; N/A
 - Number:
 - Size:
 - Skin involvement:
 - Distance from margins: N/A
 - Lymph nodes number and size: N/A
 - Additional pathologic findings: N/A
 - Immunohistochemistry technique:

Microscopic examination:

- Histological type: (1) Right breast; Invasive duct carcinoma grade II. (2) Left breast, core biopsy; intramammary lymph node with reactive changes.
- Angioinvasion: N/A
- Ductal carcinoma in situ: N/A
- Tumor extent N/A
 - Number
 - Size
 - Skin involvement:
- Lymph node involvement: N/A
- Resection Margins: N/A
 - Free /DCIS/ invasive carcinoma
- Additional pathologic findings:

Diagnosis:

Bilateral breast masses, Core biopsies;

- Right breast, Core biopsy; Invasive duct carcinoma grade
- St (ER, PR, Her2, Ki67) pending.
- Left breast, Core biopsy; Intramammary lymph node with reactive change.

Prof. Dr. Lubna Omar.

Dr. Mona Mohamed

Patient Name : Mrs. / جميله على حسين جاد الكريم
Patient No. : 25712072701644
Path No. : 2402/22
Sex : Female
Age : 64 Y
Referring Doc :

Request No. : 13006407
Request Date : 20/08/2022
Verification Date : 30/08/2022
Financial Category : نفقة الدولة - ار السلام
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

Clinical Data:
Right breast cancer. Post chemotherapy.

Nature of the specimen:
MRM.

Gross Examination:

- Procedure: Left MRM specimen measured 24x20x5cm, covering skin measured 22x12cm. Nipple was unremarkable grossly. Separately received in the same container 2 fibrofatty tissue pieces measured 5x3x2cm & 4x2x2cm.
- Tumor extent: A rubbery greyish white tumour bed site.
 - o Size: 3.5x3.5x1.5cm.
 - Distance from margins: 2.5cm away from deep margin.
 - Lymph nodes number and size: 12 LNs, up to 2x1cm.

Microscopic examination:

- No remnants of tumour tissue in a previously diagnosed IDC GII.
- Ductal carcinoma in situ: No
- Lymph node involvement: All LNs were free (0/12).
- Resection Margins: Free deep margin.
 - o Free nipple & areola.
- Histologic response to therapy: Pathological complete response.
- Additional pathologic findings: No.

Diagnosis:

Right breast cancer; MRM:
-No remnants of tumour tissue in a previously diagnosed IDC GII.
-All LNs were free (0/12).

Verified by :
abdelrahman ibrahim

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Page 1 of 2

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report

[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name : Mrs. / جميله على حسين جاد الكريم
Patient No. : 25712072701644
Path No. : 2402/22
Sex : Female
Age : 64 Y
Referreing Doc :

Request No. : 13006407
Request Date : 20/08/2022
Verification Date : 30/08/2022
Financial Category : نفقة الدولة - ز السلام
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

- Free deep margin, nipple & areola.
- Pathological complete response.
- Stage: ypT0, ypN0

Prof. Dr. Gamal Abdelaty
Dr. Abdulrahman Ibrahim

Verified by :
abdelrahman ibrahim

Printing Date : 02/11/2023 10:21:46

Page 2 of 2

Printing By : .marwa moh



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report

[[DOCTYPE:PATHOLOGY_REPORT]]



Patient Name: Gamila Ali Hussein Gad Al Karim	Date : 25/11/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 67 Year(s)
Medical ID: 2420/2021	Pat. ID: 11547

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- ⚡ **Reported case of breast cancer**
- ⚡ The quality of examination is degraded by patient's inevitable motion artifacts.
- ⚡ As Compared to the previous study dated 21/07/2022, The current study shows:

1. Chest:

- Evidence of right MRM with clear operative bed.
- Few prominent **left** axillary level III lymph nodes, no other pathologically enlarged axillary groups lymph nodes for sonomammographic correlation.
- **Newly developed** right upper and middle lung lobes reticulations likely post radiation pneumonitis for clinical correlation.
- Still noted lingular thick atelectatic bands.
- No evidence of pulmonary masses or cavitary lesions.
- **More appreciated** being more dense in today's study enlarged retrocaval, prevascular, pretracheal and paratracheal mediastinal lymph nodes with largest at retrocaval still measures about 1.8x1.3 cm.
- No evidence of pleural effusion, thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.



Patient Name: Gamila Ali Hussein Gad Al Karim	Date : 25/11/2024
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	Age : 67 Year(s)
Medical ID: 2420/2021	Pat. ID: 11547

- **Newly developed** right superior and inferior pubic ramus comminuted fractures reaching inferior aspect of right acetabulum with suspected soft tissue component/granulation Tissue ? **pathological fracture.**
- Bilateral sacroiliitis and vacuum phenomenon showing **Newly developed** diffuse ill-defined sclerotic texture of their articular surfaces on both sides more on left associated with **suspected** non displaced inferior sacral alae thin fissure line reaching articular surfaces in today's study, for bone scan assessment.
- Otherwise the scanned skeleton shows dorsal DISH, no definite worrisome lytic or sclerotic lesions.

OPINION:

- **Right MRM with clear operative bed and few prominent left axillary level III lymph nodes for sonomammographic correlation.**
- **Newly developed right upper and middle lung lobes radiation pneumonitis.**
- **Rather stationary course being more dense regarding the mediastinal lymph nodes.**
- **Newly developed right pubic bone and suspected both sacral alae fractures ? post traumatic ? pathological fractures as described for bone scan assessment.**
- **No definite hepatic metastatic deposits.**

Dr. Shaimaa Khaled, MSc.

Dr. Walid Mohamed, M.D.

8/30/26, 11:52 AM

code:02 -- Name_A MDT -- Name_E MDT

		مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية	
			08/30/2026
MDT FORM			
Name: Gamila Ali Hussein //		MRN: 7280	
Age: 69 Y		ID: 25712072701644	
Menopausal Sta POST		Phone: 01122104010	
NRS SCORE: PS 2		Marital Status:	
BMI:		Family History: -VE	
CO Morbidity: DM + HTN		Reason Of MDT: MBC	

Clinical Data:

BX:

ER: 8/8 PR: 6/8 HER2: +1 KI67:

R

L

Sonomammography:

8/2021 POST HORMONAL TTT NEO ADJUVANT

RT MRM

ADJUVANT :FEMARA + PORTH

BREAST CANCER ON HT AI SINCE 2021 TILL NOW

-PT DEVELOPED LT SUPRACLAVICULAR LN

-6/2026 BIOPSY TRU CUT BIOPSY SHOWED:

POSITIVE METASTATIC DEPOSITS OF BREAST ORIGIN GRADE 3

IPT : ER 8/8 PR 6/8 HER2 +1 KI67 25%

Mets Workup:

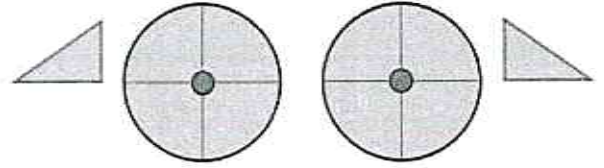
8/2026 CT CAP :

1/MEDISTINAL ,TRACHEAL ,CARINAL,UBCARINAL LN

2/BONE METS

History:

2ND LINE



TNM

Recommendations:MDT Outcome & Plan

for faslodex/ibrance

zometa every 3 ms

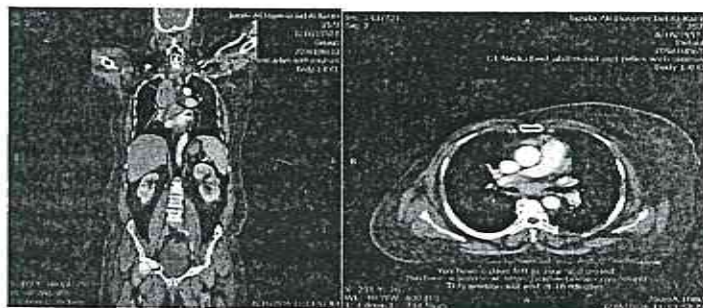
SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت	د. أحمد نجيب				

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Patient's Name: Jamila Ali Hussein Jad Al-Karim
Date: 2026-08-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE NECK, CHEST, ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- No abnormally-enhancing pathologically enlarged cervical lymph nodes.
- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.
- Normal left breast.
- **Benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 28mm.**
- Enhancing anterior mediastinal, pretracheal, left peri-vascular, carinal, subcarinal pathologically-enlarged lymph nodes are seen with the largest group seen in the pretracheal group and measure about 77x48mm in dimensions
- Mild reticulations are seen in the anterior segment of right upper pulmonary lobe, in the middle lobe and in the lingula.
- Thin linear atelectatic bands are seen in both lower pulmonary lobes.
- No sizable abnormally-enhancing pulmonary nodules, other patchy areas of consolidations or foci of calcifications noted.
- Patent central airways.
- No pleural or pericardial effusions. Yet, minimal bilateral pleural thickening is seen, more on the left side.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.** An osteolytic lesion is seen at the junction between the right acetabulum and



Patient's Name: Jamila Ali Hussein Jad Al-Karim
Date: 2026-08-16
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

the posterior end of right superior pubic ramus with enhancing soft tissue component ,measuring about 35x30mm in dimensions.

- Average size of the liver, measuring about 15.2cm, showing diffuse reduced parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.2 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- Evident right mastectomy with no evidences of either residual/recurrent neoplastic lesions seen at the operative bed.
- Enhancing anterior mediastinal,pretracheal ,left peri-vascular,carinal,and subcarinal pathologically-enlarged lymph nodes with an osteolytic lesion at the junction between the right acetabulum and the posterior end of right superior pubic ramus with enhancing soft tissue component ,likely to be metastatic in nature.
- Mild reticulations in the anterior segment of right upper pulmonary lobe,in the middle lobe and in the lingula with thin linear atelectatic bands in both lower pulmonary lobes and minimal bilateral pleural thickening ,more on the left side,likely to be post therapeutic sequelae.

Prof/Hanan El-Ahwal.

جوسلف
روستام
مدير



مبادرة رئيس الجمهورية
لدم صحة المرأة المصرية

الهيئة العامة
للصحة
مصر

08/30/2026

MDT FORM

Name:	Gamila Ali Hussein / / / / /	MRN:	7280
Age:	69 Y	ID:	25712072701644
Menopausal Sta	POST	Phone:	01122104010
NRS SCORE:	PS 2	Marital Status:	
BMI:		Family History:	-VE
CO Morbidity:	DM + HTN	Reason Of MDT:	MBC

Clinical Data:

BX:

ER: 8/8

PR: 6/8

HER2: +1

KI67:

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L

Sonomammography:

8/2021 POST HORMONAL TTT NEO ADJUVANT

RT MRM

ADJUVANT : FEMARA + PORTH

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POSITIVE METASTATIC DEPOSITS OF BREAST ORIGIN GRADE 3

IPT : ER 8/8 PR 6/8 HER2 +1 KI67 25%

Mets Workup:

8/2026 CT CAP :

1/MEDISTINAL , TRACHEAL , CARINAL, UBCARINAL LN

Z/BONE METS

History:

ZND LINE

Recommendations:

MDT Outcome & Plan

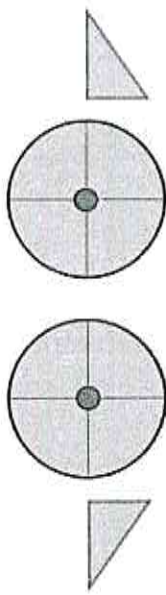
for fastodex/ibrance

zometa every 3 ms

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. مينا عورت	د. احمد لطيف				

T N M



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Patient's Name:

Jamila Ali Hussein Jad Al-Karim

Date:

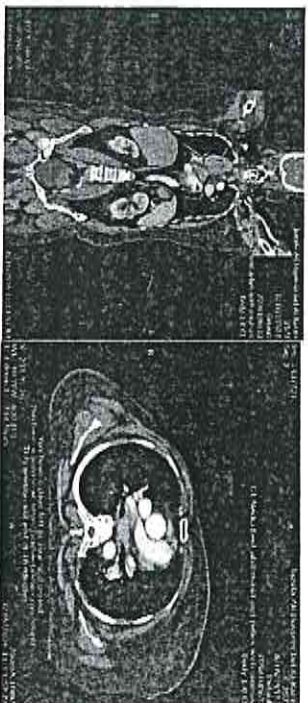
2026-08-16

Referred by:

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Hospital:

Goustave Roussey International Egypt



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WITH CONTRAST ADMINISTRATION REVEALED:-**

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- Thin linear atelectatic bands are seen in both lower pulmonary lobes.
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- Patent central airways.
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- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.** An osteolytic lesion is seen at the junction between the right acetabulum and

Patient's Name:

Jamila Ali Hussein Jad Al-Karim

Date:

2026-08-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

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IMPRESSIONS:

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- Enhancing anterior mediastinal,pretracheal ,left peri-vascular,carinal,and subcarinal pathologically-enlarged lymph nodes with an osteolytic lesion at the junction between the right acetabulum and the posterior end of right superior pubic ramus with enhancing soft tissue component ,likely to be metastatic in nature.
- Mild reticulatons in the anterior segment of right upper pulmonary lobe,in the middle lobe and in the lingula with thin linear atelectatic bands in both lower pulmonary lobes and minimal bilateral pleural thickening ,more on the left side,likely to be post therapeutic sequelae.

Prof/Hanan El-Ahwal.

جوسلف
روستام
مدير



مبادرة رئيس الجمهورية
لدم صحة المرأة المصرية

الهيئة العامة
للصحة
مصر

08/30/2026

MDT FORM

Name:	Gamila Ali Hussein / / / / /	MRN:	7280
Age:	69 Y	ID:	25712072701644
Menopausal Sta	POST	Phone:	01122104010
NRS SCORE:	PS 2	Marital Status:	
BMI:		Family History:	-VE
CO Morbidity:	DM + HTN	Reason Of MDT:	MBC

Clinical Data:

BX:

ER: 8/8

PR: 6/8

HER2: +1

KI67:

R

L

Sonomammography:

8/2021 POST HORMONAL TTT NEO ADJUVANT

RT MRM

ADJUVANT : FEMARA + PORTH

BREAST CANCER ON HT AI SINCE 2021 TILL NOW

-PT DEVELOPED LT SUPRACLAVICULAR LN

-6/2026 BIOPSY TRU CUT BIOPSY SHOWED:

POSITIVE METASTATIC DEPOSITS OF BREAST ORIGIN GRADE 3

IPT : ER 8/8 PR 6/8 HER2 +1 KI67 25%

Mets Workup:

8/2026 CT CAP :

1/MEDISTINAL , TRACHEAL , CARINAL, UBCARINAL LN

Z/BONE METS

History:

ZND LINE

Recommendations:

MDT Outcome & Plan

for fastodex/ibrance

zometa every 3 ms

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. مينا عورت	د. احمد لطيف				

T N M

